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February 19, 2026

The Honorable Robert F. Kennedy, Jr.
Department of Health and Human Services
Attention: Request for Information: HHS Health Sector AI RFI
Mary E. Switzer Building
Mail Stop: 7033A, 330 C Street SW
Washington, DC 20201

Dear Secretary Kennedy,

The American Academy of Dermatology Association (AADA) writes to provide comments on the Department of Health and Human Services (HHS) [Request for Information \(RFI\): Accelerating the Adoption and Use of Artificial Intelligence \(AI\) as Part of Clinical Care](#). The AADA represents more than 17,500 dermatologists nationwide who are committed to excellence in the medical and surgical treatment of skin disease; advocating for high standards in clinical practice, education, and research in dermatology and dermatopathology; and driving continuous improvement in patient care and outcomes while reducing the burden of disease.

AADA supports thoughtful use of AI provided its design enables reliable, clinically accurate, and high-quality patient care without overburdening physicians. We urge HHS to implement ongoing evaluation to ensure these tools maintain quality and effectiveness. We also ask HHS to consider financial barriers to implementing AI systems as well as the impacts of AI on diagnostic and utilization management tools such as claims, coding, and prior authorization review tools as outlined below. We appreciate the opportunity to contribute to this important dialogue with HHS and offer the following comments in response to the questions in the RFI.¹

Barriers to Private-Sector AI Innovation and Clinical Adoption

As HHS seeks to identify barriers to AI innovation in clinical care, the agency must ensure these tools never supplant or restrict a patient's ability to access in-person care when they want it. The Academy supports

¹ "Where have AI tools deployed in clinical care met or exceeded performance and cost expectations and where have they fallen short? What kinds of novel AI tools would have the greatest potential to improve health care outcomes, give new insights on quality, and help reduce costs?" [Federal Register :: Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care](#)

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patient access to physician led care and the freedom to choose their own physicians, including specialists and subspecialists. Physicians must also be able to access and implement AI should they choose to do so. Yet the financial cost to practices seeking to implement AI may be prohibitive, particularly to smaller, rural practices. Physicians should not be penalized if they do not have the resources to incorporate AI into their practice. HHS is encouraged to develop guidance to promote AI affordability while still accounting for necessary oversight of AI by clinical experts, as discussed in greater detail below.

Regulatory, Payment Policy, and Programmatic Design Considerations of AI in Clinical Care

AADA requests that HHS promote guardrails that support the delivery of high-quality care to patients and incorporate the following principles:

- Sensitive data used to train AI algorithms must be handled in accordance with patient privacy laws and ethical standards,
- AI algorithms must be developed using auditable data that is representative of the population on which it will be used,
- AI algorithms used for the delivery of patient care must be subject to rigorous and transparent data validation, routine quality and safety evaluation to assess model drifts, and ethical review before deployment,
- the algorithms themselves must also be transparent, accessible, and comprehensible for the intended end user(s), and
- Design, implementation, and evaluation of AI tools for clinical care should responsibly incorporate input and feedback from key stakeholders, including physicians and patient advocates.

The Academy offers the following perspectives on payer-developed AI programs in response to HHS's request for comments on AI performance in clinical care. The Academy is specifically focused on the impact payer AI-enhanced utilization review programs, such as claims review, coding, and prior authorization programs, have on clinical care and payment.

AI Payer Claims Review Programs

Major health insurers have deployed AI programs to expedite review of physician-submitted claims, which has the potential to promote more timely reimbursement. Yet dermatologists report an increasing number of payers unilaterally adjusting reported Evaluation and Management (E/M) code level (downcoding) claims and reducing payment automatically using AI tools, without first requesting and reviewing clinical records that are integral to the adjudication of the claim. Increasingly, these payer AI programs are operating without notice to dermatologists. Improper downcoding destabilizes accurate data collection and analyses regarding patient encounters, undermines practices' financial stability, and wastes resources on appealing downcoded claims. The AADA opposes improper payment reductions for services appropriately documented as it creates unnecessary administrative burden for practices forced to appeal improperly downcoded claims. Further,

payer use of AI adjusted risk scores in quality payment programs need to incorporate appropriate attribution methodology based on valid, meaningful and transparent quality metrics.

Beyond the impact on physicians, human review is necessary to ensure coding accuracy and prevent inappropriate downcoding and denials, which negatively impact patient care. The nuance and complexity of medical records demand careful judgement and a thorough contextual understanding that only human expertise can provide when determining the correct procedure modifiers and codes. Without human expertise, AI systems may misinterpret nuanced or complex medical records. These challenges underscore the importance of skilled human coders working in tandem with AI tools, allowing automation to support rather than replace human expertise and patient-centered clinical knowledge. Flagged claims should be reviewed by healthcare professionals with relevant expertise examining patient records. Furthermore, insurers should be required to track and report the accuracy of their AI claim review programs to HHS; AADA recommends that HHS audit insurers whose AI claim review programs raise suspicions of misuse. While AI offers the potential to streamline claims processing and timely payment, over-reliance on the technology by payers results in real and—at times—irreversible harm to patients in the form of delayed or inaccessible care. Payer use of AI therefore warrants special scrutiny to preserve the integrity of clinical decision-making and ensure timely access to medically necessary care.

We recommend that HHS enhance its regulatory oversight of AI-driven payer downcoding and develop guidelines for best practices on using AI in claims review. Recommendations for best practices include using AI as the first step to review and flag claims suspected of inappropriate coding, followed by appropriate human examination, and ensuring transparency and ongoing evaluation of AI in claims review.

Use of AI in Prior Authorizations

Poorly designed payer-driven prior authorization programs increase direct and indirect costs for physicians and patients alike, resulting in delays in care, adding administrative burden to practices, and amplifying patient stress. Novel AI tools have the potential to alleviate the burdens associated with payer-driven prior authorization programs that physicians experience. For example:

- Ambient intelligence uses AI in devices to detect the presence of humans and respond accordingly. This type of AI tool can be used to populate dermatology clinical notes, capture patient data from notes, and automatically complete prior authorization requests.
- Large language models can analyze clinical notes and images to strengthen prior authorization requests.
- AI-powered clinical decision support tools help ensure that submissions meet payer-specific criteria, and automated systems can generate comprehensive prior authorization narratives based on patient records.

However, until AI systems are trained on clinical data derived from medical records - rather than public data - and can accurately recognize complex medical care, payers' clinical staff must continue to review prior

authorization denials. **To address poorly designed payer-driven prior authorization programs and support physicians, we recommend HHS take the following actions:**

- Implement public reporting requirements on payers' use of prior authorization to HHS, including approval and denial rate by each service or medication. In addition to making payer prior authorization more transparent, this would also enable AI systems to learn from and enhance the review process. In 2025, major insurers met with Secretary Kennedy and Administrator Oz to pledge their commitment to streamline and improve prior authorization processes.² The Academy applauds Secretary Kennedy and Administrator Oz for securing this unprecedented commitment and urges the implementation of reporting requirements to ensure adherence to that commitment.
- Direct insurers to use AI solely for approving prior authorizations while requiring their clinical staff to review denials.
- In coordination with AI entities, offer grants to small, independent practices seeking to implement AI systems to facilitate adoption of practice-based systems to relieve the administrative burden in complying with prior authorization requirements.

AI in the Practice Setting: Physician Coding and Diagnostic Tools

Similar to recommendations for payer AI use, the AADA recognizes potential benefits for dermatology practices to integrate use of AI-driven coding and diagnostic tools, while continuing to encourage inclusion of human examination in coding and diagnostics. Using AI tools to automate routine tasks such as code identification and compliance checks with billing guidelines empowers dermatologists and their coding staff to increase successful claims. If used appropriately, shifting to AI automation of routine tasks enables a stronger focus on improving and streamlining medical record documentation, which enhances AI accuracy in code selection. The result is fewer coding errors, more timely claim submissions, and a smoother revenue cycle — contributing to greater operational efficiency and financial stability. Additionally, physicians may consider incorporating AI into diagnostic tools to improve efficiency, reduce tedium, and perform triage. However, practices should consider AI as an assistant, not a replacement to human review of diagnostic tools. While AI may eventually produce tools capable of aiding in diagnostics, caution is urged.

In contrast to payer use of AI, which too often results in barriers to care, physicians are adopting AI as a supportive tool to enhance accuracy, efficiency, and patient outcomes—and not to replace critical human-in-the-loop oversight. Any regulatory policies should therefore aid physician access and avoid imposing additional compliance burdens so physicians can continue to focus on patient care rather than administrative complexity.

We encourage HHS to consider the following recommendations:

- Most AI systems are trained using public data, which introduces accuracy and privacy issues. These issues range from HIPAA adherence when using sensitive data, presence of inaccurate or outdated information, and increased risk of errors due to the complexity of claims. We encourage HHS to convene a work group of medical professionals and AI data experts to develop appropriate regulations

² [HHS Secretary Kennedy, CMS Administrator Oz Secure Industry Pledge to Fix Broken Prior Authorization System | HHS.gov](#)

on AI training and privacy. Dermatologists should be a key part of this group, given their work across a wide range of services from routine procedures to highly specialized treatments that require precise data.

- Develop guidelines for best practices on using AI diagnostic tools. Recommendations for best practices include using AI as the first step to review diagnostics and flag where additional human expert review is required before a clinical decision is made.
- To ensure physicians have access to accurate AI-driven coding tools, we recommend HHS institute post-marketing surveillance of these tools to ensure safety. Routine evaluation of AI-driven coding and diagnostic tools will ensure the technology evolves in a manner that protects patients, improves quality, cost, and effectiveness of care delivery.
- AADA recommends that HHS, in coordination with AI entities, provide research grants focused on evaluating the clinical effectiveness and patient safety of AI tools. Funding research on AI in clinical care benefits AI-driven coding and diagnostic tools. Research will help protect patients, promote clinical effectiveness, and determine how data collected during model deployment can be used to retrain and refine models for validation studies and post deployment monitoring. Ultimately, research findings may be used to enhance AI's potential to improve health care outcomes, give new insights on quality, and help reduce costs.
- AADA recommends that HHS collaborate with other federal agencies, including the Department of Justice and Office of Inspector General, to identify regulations regarding physician liability and to issue guidance on their application when AI is used to help clarify risk allocation. AI is still developing and comes with imperfections which may result in exposing physicians to liability, including medical malpractice concerns. Additionally, using AI for coding and billing without effective oversight may result in billing errors and overpayments that have fraud and abuse implications. To address these concerns, we recommend that HHS establish a safe harbor that clarifies and limits physician liability when AI is used as a support tool in the case of documented clinical oversight, compliance protocols, and diagnostics. The financial cost of implementing AI systems is a deterrent for independent physician practices. To implement AI-driven systems, a practice must endure costs associated with technology acquisition, integration, and staff training. Additionally, the potential risks of implementation — such as system errors, underperformance, or failure to meet expectations — can deter practices from adopting these technologies without clear evidence of return on investment. We encourage HHS to, in coordination with AI entities, offer federal and state grants to small, independent practices seeking to implement AI systems.

Conclusion

We applaud HHS' focus on accelerating adoption and use of AI in clinical care. AI tools present an opportunity to improve health care outcomes, give new insights on quality, and help reduce costs. These tools also have the potential to increase administrative burdens on practices, delay care, and contribute to practices' financial instability. We encourage HHS to thoroughly consider the recommendations provided in this letter and appreciate the opportunity to offer comments. If you have any questions regarding this letter, please contact Lou Terranova, Associate Director, Health Policy & Payment, at lterranova@aad.org or 847-240-1465.

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Sincerely,

A handwritten signature in cursive script that reads "Susan C. Taylor MD, FAAD".

Susan C. Taylor, MD, FAAD
President, American Academy of Dermatology Association